

Durable Medical Equipment (DME)

Non-clinical medical

Durable Medical Equipment.

Equipment and supplies ordered by a health care provider for everyday or extended use. Coverage for DME may include: oxygen equipment, wheelchairs, crutches or blood testing strips for diabetics.

Many insurance plans provide coverage for DME, including Medicare Part B. To qualify as DME under Medicare, a piece of equipment or device must have an expected lifespan of about 3 years with repeated home use and serve a strictly medical purpose.

Expendable medical supplies such as bandages, masks, and rubber gloves are not considered DME under Medicare.



Importance of D.M.E

Durable medical equipment (DME) enables patients with a debilitating medical condition, illness or injury to achieve a higher quality of life while living at home.

DME may be ordered by a healthcare provider to accommodate a patient's temporary disability, as in the case of a wheelchair or crutches for a recovering patient, or as part of a patient's long-term care plan for their chronic or life-limiting condition, such as a mechanical ventilator for a patient with advanced lung disease.



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As interpreters we will be interpret some basic medical calls for **correctional facilities**. We will use the same interpreting protocol as we already do.

You may occasionally interpret a call from a correctional medical facility, or from a detention facility or from the **Customs and Border Protection**.

These interactions will involve a health care provider, such as doctor or a nurse, and an **inmate** in custody.

At times, other parties may be present, such as **prison guards** or **correctional officers** (CO) to protect the security of the provider.

When an inmate patient requests a sick bay visit, the healthcare provider needs to take the medical history and some areas are addressed during these calls, for example:



- General intake questions
- Medical History
- STD's (chlamydia, genital warts, genital herpes, gonorrhea, (among the treatable ones), but also Hepatitis B & C, syphilis and HIV.
- Tattoos or scars
- Mental health services



Hospice Care

Non- clinical medical

Hospice care helps people **nearing the end-of-life** who may have six months or less of life.

The idea is to maximize comfort for a person who is **terminally ill** by reducing pain and addressing physical, psychological, social and spiritual needs.

The goal of hospice care is to **support the highest quality of life possible** for whatever time remains.



Palliative Care

Palliative care focuses on providing patients relief from pain and other symptoms of a serious illness, regardless the diagnosis or stage of the disease.

Aims to **improve quality of life** for both patients and their families.

It is offered alongside curative or other treatments they might be receiving.



Palliative Sedation

Non- clinical medical

The practice of **sedating** a terminally ill patient to the point of unconsciousness, due to unmanageable pain and suffering that has been resistant to traditional medical management. Such patients are imminently dying, usually hours or days from death.

Palliative sedation may occur for a short period, offering the patient respite from their pain, or the patient may be sedated until s/he dies. This practice does not accelerate the process of death.



Legal documentation

Advanced Directives

Documents used by doctors and caregivers when an individual is terminally ill, has experienced a life-threatening injury, is in a coma or near the end of life. These directives remove the burden of uncertainty that most family members experience when difficult medical decisions must be made. The three most common AD are:

Legal Power of Attorney	DNR Order	Living will
Document where an individual names one person to make decisions for them when they are unable to do so themselves.	A do-not-resuscitate order is a medical order written by a doctor. It instructs health care providers not to do CPR if a patient's breathing stops or if the patient's heart stops beating.	Legal written document with instructions regarding an patient's preference for medical care when they are unable to make decisions for themselves.